

Beyond Biology: How Stigma continues to shape HIV/AIDS treatment

HIV is a greedy, cunning disease: it isn't enough for it to just latently lurk in a patient's cd4 cells for decades:¹ It further debilitates by evading the body's defenses through constant mutations, always staying one step ahead.² However, one could argue that these biological mechanisms are eclipsed by the devastating social and cultural factors surrounding HIV; Indeed, despite ART effectively quenching HIV in the body by suppressing replication to undetectable levels and completely preventing transmission (aka Viral Suppression- all for just one pill a day), the HIV epidemic still continues- over 30,000 people acquired HIV in 2021, when data was last collected.³ Biology clearly can't encapsulate everything when it comes to explaining why this disease continues to spread.

So why do people continue to suffer from HIV? In Dr. Claire Farel's reflection on her experience with a patient in prison who she watched wither away, while his HIV meds sat untouched, she states:

“when medications exist that can literally make the difference between life and death—sometimes in days—it's easy to focus on the pills and how to get them. But the problem is bigger than medication, and it is not solved with a white coat and a stethoscope...the burden of HIV-related stigma makes a task as seemingly straightforward as calling a free clinic for an appointment an insurmountable challenge for many people.”⁴

¹ Fromentin and Chomont, “HIV Persistence in Subsets of CD4+ T Cells.”

² Cuevas et al., “Extremely High Mutation Rate of HIV-1 In Vivo.”

³ “Basic Statistics | HIV Basics | HIV/AIDS | CDC.”

⁴ Farel, “Water, Water, Everywhere.”

This quote highlights the insufficiency of a biological solution when stigma can compromise an individual's abilities to connect with care or maintain their treatments. Thus, improving patient outcomes for people living with HIV (PLWHIV) must include tackling this stigma. Stigma leads to discrimination, prejudice and stereotypes, which can pose many challenges to an individual. In an effort to avoid this stigma and discrimination, patients are disincentivized from seeking a diagnosis or care in the hopes that this will prevent anyone else from finding out.⁵ Unfortunately, the effect of this stigma and avoiding treatment consigns individuals to suffer from a treatable infection and potentially its companion disease AIDS, which is lethal. But the consequences can be even more immediately dire: In the words of one PLWHIV who attempted suicide after their diagnoses and didn't even want to seek care, "I didn't know all the [advancements in testing and treatment] but I knew the stigma."⁶

Faced with the insurmountable stigma and prejudice associated with HIV, their response is brutally understandable, as the stigma of HIV can seem insurmountable when you consider its history and brutal depth.

Indeed- the stigma of HIV is synonymous with the stigma surrounding other minority communities. In 1981, the first case of HIV was diagnosed, but it was not called HIV yet. Without a culprit infectious disease vector to blame, it was simply associated with gay people (specifically gay men). For example, in 1982, doctors started using the "4 H's" to identify patients who were at higher risk for HIV: Homosexuals, Haitians, Hemophiliacs, and Heroin

⁵ NASTAD, "Intro to Health Equity."

⁶ Quote from Day 2 Panel: PUBH 420/720

users and the New York Times had begun calling the disease Gay Related Immunodeficiency Virus or “GRID.”⁷ Associating the disease with specific groups had dire consequences: to the American public, it seemed that only these groups were vulnerable to the disease. The stereotype became that if you were a part of one of these groups, you had HIV, and likewise that if you had HIV you must be a part of one of these groups.⁸ Thus, PLWHIV would face the assumptions that they were one of the 4 H’s, even if they weren’t. The prejudice and discrimination that the 4 groups faced (mainly homosexuals, haitians, and heroin users) was transferred to PLWHIV.

To hone in on the undesirable prejudices a PLWHIV may face for being associated with the “4 H’s”, consider the LGBTQ community. I am choosing the LGBTQ community because the first H stuck particularly well in public consciousness in terms of creating a stereotypical idea of who had HIV, as illustrated by the use of “GRID” and by actions such as banning gay men from being blood donors in 1984 (not haitians, hemophiliacs- or heroin users, just gay men).⁹ Today, prejudice against the LGBTQ community manifests as teasing, bullying, harassment, physical violence, employment discrimination, health insurance discrimination, being refused care by healthcare practitioners, housing discrimination... the list, unfortunately, goes on and on.^{10 11} HIV emerged in the 80s, on the tails of the Gay Liberation Movement and in the wake of public figures such as Anita Bryant leading the popular “save our children ” from the LGBTQ+ community campaign, along with other public figures. Moreover less legal protections existed for the LGBTQ community and public sentiment was even more against people in the sexual minority.¹² Thus, these prejudices were even greater at the beginning of the epidemic.

⁷ Hurt, “776,000 & Counting.”

⁸ CDC, “Let’s Stop HIV Together.”

⁹ Hurt, “776,000 & Counting.”

¹⁰ thisisloyal.com, “REPORT: Evidence.”

¹¹ Massey, Young, and Merriwether, “Chapter 6: Prejudice and Discrimination against LGBTQ+ People.”

¹² Hurt, “776,000 & Counting.”

Unfortunately, association with homosexuals and heroin users also created a moral stigma. The disease is spread in many ways, but one of the major ways is through anal sexual activity and needle sharing. Thus, the idea spread that it was MSM and heroin users “faults” that they contracted HIV, because they engaged in those “risky activities.” Thus, having HIV took on a moral dimension, where those who had it were to blame. The weight of being blamed for a life altering disease is incredibly heavy, and avoiding this stigma may also lead to trying to avoid diagnoses and treatment.

So, in an effort to avoid the stigma, individuals were deterred from seeking a diagnosis, or seeking care, in case others found out and the debilitating stigma ensued.¹³ In this way, stigma from being associated with the “4 H” communities and the stigma of blame impacts individuals living with HIV, and prevents individual adherence to HIV care, worsening their care outcomes.

Since a lot of the stigma stems from associations of HIV with different communities (and how that can consequently create a moral judgment), not creating this association would have done wonders. Media outlets such as the New York Times and Healthcare institutions with the ability to shape public perception should have been more aware and intentional about the association they made between communities and HIV.

Moreover, one must examine how this association between groups was undone and the stigma decreased to think of how the social response could have been improved: Patients like Ryan White (a young white hemophiliac who got HIV from a blood transfusion, “through no fault of his own”), Rock Hudson (a star, white actor player and a sex symbol for women), Magic

¹³ NASTAD, “Intro to Health Equity.”

Johnson (star basketball player) became symbols that shifted the public association of who got HIV and mobilized the government to act and pass legislation that improved outcomes for PLWHIV. Moreover, other gay men (Freddie Mercury, white and part of a beloved band) who died publically of HIV were still beloved and helped to undo some of the othering and lack of empathy for those dying of HIV.¹⁴ Public symbols did a lot to shift the association, and concerted efforts to increase representation of non “4H” groups may have destigmatized HIV, changing it from a disease of specific groups who are easy to blame to what it really is: just a disease.

¹⁴ Hurt, “776,000 & Counting.”

Bibliography:

“Basic Statistics | HIV Basics | HIV/AIDS | CDC,” May 22, 2023.

<https://www.cdc.gov/hiv/basics/statistics.html>.

CDC. “Let’s Stop HIV Together: HIV Stigma.” Centers for Disease Control and Prevention,

November 3, 2022. <https://www.cdc.gov/stophivtogether/hiv-stigma/index.html>.

Cuevas, José M., Ron Geller, Raquel Garijo, José López-Aldeguer, and Rafael Sanjuán.

“Extremely High Mutation Rate of HIV-1 In Vivo.” *PLoS Biology* 13, no. 9 (September 16, 2015): e1002251. <https://doi.org/10.1371/journal.pbio.1002251>.

Farel, Claire E. “Water, Water, Everywhere.” *JAMA* 312, no. 4 (July 23, 2014).

Fromentin, Rémi, and Nicolas Chomont. “HIV Persistence in Subsets of CD4+ T Cells: 50

Shades of Reservoirs.” *Seminars in Immunology* 51 (January 2021): 101438.

<https://doi.org/10.1016/j.smim.2020.101438>.

Hurt, Christopher. “776,000 & Counting: A Timeline of HIV in the US,” n.d.

Massey, Sean G., Sarah R. Young, and Ann Merriwether. “Chapter 6: Prejudice and

Discrimination against LGBTQ+ People.” In *Introduction to LGBTQ+ Studies: A Cross-Disciplinary Approach*. State University of New York Press. Accessed March 26, 2024.

<https://milnepublishing.geneseo.edu/introlgbtqstudies/chapter/prejudice-and-discrimination-against-lgbtq-people/>.

NASTAD. “Intro to Health Equity.” Accessed March 26, 2024.

<https://nastad.org/sites/default/files/2021-12/PDF-Intro-Health-Equity.pdf>.

thisisloyal.com, Loyal |. “REPORT: Evidence of Housing Discrimination Based on Sexual

Orientation and Gender Identity.” UCLA Williams Institute School of Law, February

2016. <https://williamsinstitute.law.ucla.edu/publications/lgbt-housing-discrimination-us/>.